Duke University Health System (Duke Health)

Overview

Duke University Health System, operating publicly as Duke Health, is a private, nonprofit academic health system headquartered in Durham, North Carolina. It is the clinical, research, and educational enterprise of Duke University, anchored by the Duke University School of Medicine and School of Nursing and integrated with a hospital network, a globally recognized clinical research organization, a translational science institute, and an expanding AI and digital health infrastructure. As a more-than-$7 billion enterprise with four hospitals, more than 140 primary and specialty care clinics across 10 counties in central and eastern North Carolina, and approximately 26,000 health system employees, Duke Health is the Triangle region's dominant academic medical system and UNC Health's most direct peer and regional competitor.

partnership structures can inform how UNC positions its own assets, and a potential collaborator on specific Triangle-based initiatives — particularly where existing inter-institutional CTSA data infrastructure and co-funded research already create documented overlap. The relationship requires care to distinguish competitive positioning from genuine alignment opportunities.

Sources: Duke Health Corporate [1]; Business North Carolina [2]; Prism News [3].

Basic Organizational Information

Field	Value
Full Name	Duke University Health System, Inc. (operating as Duke Health)

Field	Value
Headquarters	Durham, North Carolina, United States
Website	https://www.dukehealth.org
Corporate Site	https://corporate.dukehealth.org
Newsroom	https://corporate.dukehealth.org/newsmedia
Org Type	Private Nonprofit Academic Health System
Revenue	More than $7 billion (FY ended June 30, 2025)
Employees	~26,278 full-time DUHS employees; 11,000+ at the academic medical center
Hospitals	4 hospitals
Clinics	140+ primary and specialty care clinics in 10 counties
CEO	Dr. David Zaas (effective May 1, 2026)
Chief Academic Officer	Dr. Mary Klotman, EVP for Health Affairs and Dean, School of Medicine
Chief Digital Officer	Dr. Jeffrey Ferranti, SVP and Chief Digital Officer
EHR Platform	Epic (enterprise-wide; unified across hospitals and 300+ ambulatory clinics)

Sources: Duke Health Facts & Statistics [4]; Business North Carolina [2]; Duke Health Newsroom [5].

System Scale and Structure

Hospital Network

Duke Health comprises four hospitals. Duke University Hospital is the flagship academic medical center on the Duke University Medical Center campus in Durham — a research and teaching hospital consistently ranked among the nation's best. Duke Regional Hospital serves Durham, Orange, Person, Granville, and Alamance counties with a focus on community-oriented specialty care. Duke Raleigh Hospital is a campus of Duke University Hospital in Wake County, serving the greater Raleigh area. Duke Health Lake Norman Hospital, a 123-bed acute-care facility in Iredell County, was acquired by Duke Health in 2025 from its former operator, extending the system's reach into the greater Lake Norman area and surrounding counties.

Duke University Hospital houses Duke Children's Hospital and Health Center, giving the system a dedicated pediatric inpatient and subspecialty platform within its flagship campus.

Ambulatory and Primary Care

Duke Health Integrated Practice (DHIP), formerly the Private Diagnostic Clinic, operates more than 140 primary and specialty care clinics in 10 counties in central and eastern North Carolina. Duke Primary Care is the largest primary care network in the greater Triangle area, with family medicine, internal medicine, pediatrics, and urgent care providers at approximately 50 locations. Duke Connected Care is a community-based, physician-led network coordinating care for Medicare Fee-for-Service patients in Durham and surrounding counties.

Home and Wellness Services

Duke HomeCare & Hospice provides hospice, home health, and infusion services through two inpatient hospice facilities in Durham and Hillsborough. Duke Health & Well-Being encompasses medically-based fitness, weight management, rehabilitation, and integrative medicine programs through the Duke Health & Fitness Center and Duke Integrative Medicine.

Sources: Duke Health System Overview [1]; Duke Health Facts & Statistics [4]; Prism News [3].

Quality, Rankings, and Accreditation

U.S. News Best Hospitals 2025–26

Duke University Hospital was ranked No. 1 in North Carolina and No. 1 in the Raleigh-Durham region in the U.S. News & World Report 2025–26 Best Hospitals rankings. Duke was nationally ranked in 11 adult specialties and in 10 pediatric specialties in those rankings. Duke Children's Hospital was ranked No. 1 in North Carolina and tied for No. 1 in the Southeast, with Cardiology & Heart Surgery ranked third nationally for the third consecutive year. Duke University Hospital dropped from the U.S. News Honor Roll in the 2025–26 cycle after appearing on the list in the prior year.

Leapfrog Patient Safety

Duke University Hospital has received top scores for patient safety from The Leapfrog Group for 14 consecutive grading periods — a sustained record cited by the health system in its public reporting.

School of Medicine Research Rankings

The Blue Ridge Institute for Medical Research ranked Duke University School of Medicine fourth in the nation in NIH funding for all grants and contracts in fiscal year 2023. U.S. News ranks eight specialty programs in the School of Medicine in the top 10 nationally, including Anesthesiology (third), Internal Medicine (fourth), and Surgery (fourth). The School of Nursing is ranked fourth nationally as a Best Graduate Nursing School, with its Doctor of Nursing Practice Program ranked second.

Sources: Business North Carolina [2]; Duke Children's Hospital Pediatrics [6]; Duke Health Facts & Statistics [4].

Research Infrastructure

Sponsored Research Scale

Duke is one of the largest biomedical research enterprises in the country, with more than $1 billion in sponsored research expenditures annually and more than $672 million in NIH grant funding in fiscal year 2023. The National Institutes of Health awarded Duke University School of Medicine approximately $455 million in federal funding in the most recent fiscal year reported by Business North Carolina (FY2025), a figure that reflects documented declines driven by federal funding policy changes in 2025.

Duke Clinical Research Institute (DCRI)

The DCRI is the largest full-service academic clinical research organization in the world, employing more than 1,100 faculty and staff and conducting research at more than 3,500 sites in 64 countries. The institute has enrolled more than one million patients across its studies and generated more than 8,000 publications in peer-reviewed journals. Established in 1996, the DCRI conducts multinational clinical trials, manages major national patient registries, and performs landmark outcomes research across cardiovascular disease, pediatrics, infectious disease, neuroscience, and more than 20 therapeutic areas. In 2024, the DCRI launched i-Cubed as its center for clinical research innovation. In 2025, the DCRI launched the UNICORN Network in partnership with 17 academic data coordinating centers.

Duke Clinical & Translational Science Institute (CTSI)

The Duke CTSI is the NIH-funded Clinical and Translational Science Award (CTSA) hub for Duke, catalyzing the translation of scientific discoveries into health benefits for patients and communities. Duke received one of the original 12 CTSA grants in 2006. The most recent renewal, awarded in 2025, totaled $69 million — the third and largest CTSA renewal Duke has received. The Duke CTSI is a direct partner to the UNC-RTI CTSA partnership through the Carolinas Collaborative, a data-sharing infrastructure that harmonizes EHR data across CTSA institutions at Duke, UNC, the Medical University of South Carolina, and Wake Forest University.

Duke Cancer Institute (DCI)

The Duke Cancer Institute is one of fewer than 60 NCI-designated Comprehensive Cancer Centers in the United States and has held continuous NCI recognition and funding since 1973, when it was named one of the original eight comprehensive cancer centers. The DCI encompasses more than 400 researchers and physicians, maintains clinical and research partnerships across India, Tanzania, China, Singapore, and the United States, and receives more than $145 million annually in cancer research funding. Recent CCSG renewal periods have been marked by a 47 percent increase in interventional clinical trial accruals and a 10 percent increase in therapeutic accruals.

Sources: Duke Health Facts & Statistics [4]; DCRI [7]; Duke CTSI [8]; Duke Cancer Institute [9].

Digital and AI Infrastructure

Epic Enterprise EHR

Duke Health operates on Epic as its enterprise EHR, with a system-wide implementation unified across its hospitals and more than 300 ambulatory clinics. Dr. Jeffrey Ferranti, Senior Vice President and Chief Digital Officer, directed the enterprise-wide Epic rollout. Ferranti also created the DEDUCE and DISCERN data platforms to enable secure, efficient access to enterprise research data from Duke's clinical infrastructure.

Abridge Ambient AI (January 2025)

In January 2025, Duke Health announced a strategic partnership with Abridge, the ambient clinical AI documentation platform, following a pilot that began in October 2024 with 160 physicians. Under the enterprise agreement, the Abridge platform is being deployed to 5,000 Duke Health clinicians across more than 150 primary and specialty clinics, including Duke Health Integrated Practice and Duke Primary Care locations. The deployment is integrated with Epic. Duke Health and Abridge have stated an intent to co-develop additional clinical AI applications beyond documentation, with Duke's Chief Health Information Officer Eric Poon describing the tool as shifting clinician cognitive capacity away from documentation and toward patient interaction.

Duke AI Health

Duke AI Health is a multidisciplinary initiative housed in the Duke University School of Medicine, advancing research, training, and education in AI, machine learning, and data science applications for health and healthcare. The initiative spans the School of Medicine, School of Engineering, School of Law, and multiple institutes and centers across Duke. It is directed by Michael Pencina, PhD, Vice Dean for Data Science. Duke AI Health hosts the annual Duke Summit on AI for Health Innovation (second annual held October 2025 at the North Carolina Biotechnology Center in Research Triangle Park) and the Duke Machine Learning Spring School, an in-person training program focused on generative AI methods and health applications.

Discovery AI (January 2026)

In January 2026, Dean Mary Klotman announced the launch of Discovery AI, a new strategic research initiative designed to deeply integrate AI and machine learning with fundamental biological research across the School of Medicine. Discovery AI is co-led by Scott Soderling (chair, Department of Cell Biology) and David Page (chair, Department of Biostatistics & Bioinformatics). The initiative is planning cluster hires of AI researchers embedded in at least four basic science departments, with a focus on molecular discovery and multi-scale biological modeling.

Duke Health AI Evaluation & Governance Program

The Duke Health AI Evaluation & Governance (E&G) Program, operationalized through the Algorithm-Based Clinical Decision Support (ABCDS) Oversight initiative, maintains a registry of algorithmic and AI solutions deployed in clinical care at Duke and performs structured evaluation against patient safety and quality standards. The ABCDS Oversight Committee is a joint program of the Duke School of Medicine and Duke University Health System and is directed by Nicoleta J. Economou-Zavlanos, PhD.

Duke Federated Clinical Analytics Platform (FCAP)

The Duke FCAP is a secure, protected environment allowing access to de-identified medical record data for researchers and qualified external collaborators. Duke AI Health uses the FCAP for structured educational programming, including introductory seminars co-hosted with North Carolina Central University as of May 2026.

Duke-Margolis Institute for Health Policy

Elevated from center to institute status in January 2024, the Duke-Margolis Institute for Health Policy maintains dedicated AI and Digital Health Technologies research portfolios. Duke-Margolis publishes policy research on AI in healthcare, value-based care, and digital health technology regulation, and hosts annual health policy conferences bringing together policymakers, academics, and industry. Duke-Margolis operates a public-private partnership model focused specifically on North Carolina and the American South.

Sources: DCRI [7]; Duke Health/Abridge Newsroom [10]; Becker’s Hospital Review [11]; Duke AI Health [12]; Duke AI Health Discovery AI [13]; Duke-Margolis Institute [14].

Strategic Themes

- More than $7 billion in health system revenue (FY2025) with improving operating margins: operating income rose from $12 million (0.3% margin) to $189.8 million (4.6% margin) through the first half of FY2026.
- New CEO Dr. David Zaas (effective May 1, 2026), returning after nearly 20 years at Duke — stated priority on expanding impact, strengthening the academic mission, and positioning DUHS as a national standard-setter.
- Epic enterprise EHR as unified clinical and research data platform across hospitals and 300+ clinics, enabling large-scale real-world data access.
- Abridge ambient AI deployed to 5,000 clinicians (January 2025) with co-development partnership, alongside a structured AI governance framework through ABCDS Oversight.
- Duke AI Health as the system-wide AI research and education infrastructure, with the Discovery AI initiative expanding AI into fundamental biological sciences as of January 2026.
- DCRI as the world's largest academic CRO — the most operationally capable clinical research infrastructure at any academic health system in the Triangle.
- Duke CTSI, including the Carolinas Collaborative with Duke, UNC, MUSC, and Wake Forest, as documented inter-institutional data infrastructure.
- Duke Cancer Institute's NCI Comprehensive Cancer Center designation and $145 million in annual cancer research funding as a durable anchor for oncology-focused partnerships.
- Duke-Margolis Institute as a policy bridge connecting clinical research, digital health technology, and federal health policy — relevant to any partnership requiring regulatory or payment model framing.
- Documented federal funding headwinds: NIH funding to the School of Medicine declined approximately $18 million per month in 2025 compared to 2024, creating potential openings for industry-funded and sponsored research relationships.

Sources: Becker's Hospital Review [15]; Duke Health Newsroom [5]; Duke AI Health [12]; Business North Carolina [2].

Alignment with UNC Research

No formal OIC-level UNC–Duke institutional partnership agreement has been identified from the current source set. The following represents documented structural overlap, shared infrastructure, and strategic alignment based on verified sources. A dedicated research pass targeting named joint grants, sponsored research agreements, and existing Carolinas Collaborative data use activities is recommended before making direct partnership claims in external communications.

Three Strategic Framing Pillars for UNC

Translational research infrastructure and inter-institutional data: The Carolinas Collaborative — a joint program of the Duke CTSI, the UNC-RTI CTSA, MUSC, and Wake Forest — already harmonizes EHR data across institutions for translational research and quality improvement. This is the most directly actionable existing infrastructure for positioning UNC to industry as a Triangle-scale data and research partner rather than a single-institution asset. Duke and UNC researchers are also actively co-leading externally funded projects — most recently a $12.9 million NIH-funded study on care for autistic children announced April 28, 2026.

AI governance and clinical evaluation infrastructure: Both Duke (through ABCDS Oversight and Duke AI Health) and UNC (through SHIRE and Epic AI co-development) have invested in structured frameworks for evaluating and deploying AI tools in clinical settings. Together, these systems represent a Triangle-scale validated AI evaluation environment — a positioning argument available for industry partners, particularly in health AI and digital therapeutics, who need multi-site academic medical center validation.

Clinical research capacity at Triangle scale: The DCRI's scale as the world's largest academic CRO and Duke's CTSI renewal create documented capacity for complex, multinational trial execution. UNC's NC TraCS and Lineberger infrastructure create complementary capabilities in biomarker-driven oncology and rural health access. Framing the Triangle as an integrated clinical research geography — rather than two competing health systems — may be relevant for large biopharma, device, and health AI partners with NC-wide or southeastern footprint requirements.

Specific UNC Research Fit Areas

- **Biomedical informatics and data science:** Duke CTSI's Carolinas Collaborative and Duke FCAP create direct data infrastructure alignment with UNC's SHIRE platform and the Carolina Health Informatics Program (CHIP). Collaborative grant proposals for multi-site EHR-based research are a defensible shared entry point.
- **Oncology and translational medicine:** Both Duke Cancer Institute (NCI Comprehensive Cancer Center) and UNC Lineberger (NCI Comprehensive Cancer Center, ARPA-H EVOLVE funding) are active in precision oncology and clinical trial expansion. Inter-institutional collaboration on trial design or patient enrollment across geographic populations is a documented model.
- **AI evaluation and responsible deployment:** Duke's ABCDS Oversight and AI Health governance programs are directly analogous to SHIRE's IRB-governed data access and Epic AI co-development. Together, the two systems represent a credible argument for a shared Triangle health AI evaluation framework relevant to industry partners developing clinical AI products.
- **Rural and community health:** Duke Connected Care and Duke's CTSI community health programs in the Triangle overlap with UNC Health's statewide rural reach and the Gillings

School's public health analytics. Federal funding opportunities targeting underserved populations frequently reward multi-institution partnerships.

- **Health policy and digital health regulation:** Duke-Margolis and any equivalent UNC health policy infrastructure represent complementary policy research capacity for industry partners navigating FDA digital health regulation, CMS coverage decisions, or value-based payment model design.

Recommended Enrichment Targets for Follow-Up Research

- UNC Carolina Health Informatics Program (CHIP) and CHIP-affiliated faculty
- NC TraCS Institute and Carolinas Collaborative staff
- UNC Lineberger Comprehensive Cancer Center — translational oncology programs
- UNC Gillings School of Global Public Health — data analytics and community health groups
- UNC Office of Innovation and Commercialization — existing Duke co-invention or licensing records
- UNC Research Computing — potential Carolinas Collaborative data access coordination

Sources: Duke CTSI Carolinas Collaborative [8]; Duke Health Newsroom [16]; Duke AI Health [12].

Talking Points

Organizational Context

- Duke Health is a more-than-$7 billion academic health system — the Triangle's leading academic medical center and the state's top-ranked hospital system by U.S. News for 2025–26.
- The health system employs approximately 26,000 people across four hospitals and more than 140 clinics in 10 counties, with the combined Duke University and health system workforce approaching 44,500 employees statewide.
- Duke University Hospital is ranked No. 1 in North Carolina by U.S. News, with 11 adult and 10 pediatric specialties nationally ranked. Leapfrog has awarded Duke top patient safety scores for 14 consecutive grading periods.
- Duke Health is a direct competitor to UNC Health in the Triangle market; competitive positioning requires care in distinguishing areas of shared research infrastructure from clinical market competition.

Research and Innovation Relevance

- The DCRI is the largest academic clinical research organization in the world — a fact that should be acknowledged directly in any framing that positions UNC's NC TraCS and Lineberger as competitive alternatives. The two systems' research infrastructures are complementary by geography and specialty, not equivalent in scale.
- Duke's $69 million CTSI renewal (2025) and the Carolinas Collaborative represent the most directly actionable mechanism for joint UNC-Duke research positioning with industry.
- The AI infrastructure stack at Duke — Abridge enterprise deployment, Discovery AI, Duke AI Health, ABCDS Oversight, and FCAP — positions Duke as one of the more operationally advanced academic health systems for clinical AI development and validation in the Southeast.

Alignment with External Partners

- For industry partners in clinical AI: Duke and UNC together represent a Triangle-scale, dual-system clinical validation environment with documented AI governance structures at both institutions. Neither system alone captures the geographic or patient population breadth that a joint positioning can offer.
- For biopharma and device companies: The DCRI's global trial execution capacity and Duke Cancer Institute's NCI designation, combined with UNC Lineberger's ARPA-H funding and rural patient reach, represent a complementary oncology research geography.
- For cloud and data infrastructure companies: The Carolinas Collaborative EHR data harmonization program and both institutions' CTSA-funded translational science infrastructure create a documented foundation for data science, real-world evidence, and applied AI partnerships.

Risk and Caution Notes

- Do not cite the $7 billion revenue figure as audited without attribution; the figure is reported by Business North Carolina (2026) based on Duke University Health System financial reporting for fiscal year ended June 30, 2025. The figure refers to the health system alone, not the combined Duke University and health system enterprise.
- Duke Health is a direct market competitor to UNC Health in the Triangle; any framing of Duke as a "partner" in external partnership materials must be scoped precisely to specific research or data infrastructure programs and must not be read as characterizing the clinical or competitive relationship.

- Dr. David Zaas became CEO on May 1, 2026 — effective at the time of this report's preparation. Confirm current leadership contacts before any formal outreach; prior CEO Craig Albanese departed September 2025.
- Federal funding pressure at Duke is documented and material: NIH funding to the School of Medicine declined roughly $18 million per month in 2025 versus 2024. This creates potential openings for industry-funded research relationships but also introduces institutional financial uncertainty that should be monitored.
- The Carolinas Collaborative is a CTSA-funded data infrastructure program, not a formal research partnership agreement between UNC and Duke. Its existence does not imply OIC-level or sponsored research-level joint activity.
- Do not overstate DCRI co-investigation possibilities without a dedicated search for existing or prior UNC-DCRI sponsored research records.

Expected Report Angle and Positioning

Positioning Summary: Duke Health should be framed as (1) a more-than-$7 billion private nonprofit academic health system — the Triangle's dominant academic medical center with four hospitals, 140-plus clinics, and approximately 26,000 employees, ranked No. 1 in North Carolina by U.S. News for 2025–26; (2) home to the world's largest academic clinical research organization in the DCRI, with a $69 million CTSA renewal and documented global trial execution capacity; (3) a leading clinical AI adopter with an enterprise Abridge deployment reaching 5,000 clinicians, structured AI governance through ABCDS Oversight, and the January 2026 Discovery AI initiative integrating AI with fundamental biological research; (4) a direct Triangle peer to UNC Health whose CTSA-funded Carolinas Collaborative infrastructure creates documented inter-institutional data access overlap with UNC — the most defensible starting point for any joint UNC-Duke positioning with external partners; and (5) a competitor and potential collaborator simultaneously, requiring precise scoping of any co-positioning claims to specific programs (CTSI data infrastructure, co-funded grants, oncology geography) rather than the clinical enterprise as a whole.

References

[1] Duke Health — Health System Overview — https://corporate.dukehealth.org/clinical-care/health-system-overview

[2] Business North Carolina — North Carolina's Best Hospitals 2026 (March 2026)

[3] Prism News — Duke University Health System Names David Zaas as CEO (April 2026)

[4] Duke Health — Facts & Statistics — https://corporate.dukehealth.org/who-we-are/facts-statistics

[5] Duke Health Newsroom — Duke Appoints David Zaas as Health System CEO (March 25, 2026)

[6] Duke Children's Hospital — Ranked No. 1 in North Carolina & Southeast (U.S. News 2025–26)

[7] Duke Clinical Research Institute — About DCRI — https://dcri.org/about

[8] Duke Clinical & Translational Science Institute — CTSA Services and Carolinas Collaborative

[9] Duke Cancer Institute — About DCI — https://www.dukecancerinstitute.org/about-dci

[10] Duke Health Newsroom — Duke Health and Abridge Partner (January 2025)

[11] Becker's Hospital Review — Duke Health, Abridge partner for clinical ambient AI (January 2025)

[12] Duke AI Health — About Duke AI Health — https://aihealth.duke.edu

[13] Duke AI Health — Discovery AI Initiative Unveiled (February 2026)

[14] Duke-Margolis Institute for Health Policy — Digital Health & AI Portfolio

[15] Becker's Hospital Review — Duke Health raises operating margin (February 2026)

[16] Duke Health Newsroom — Duke & UNC Researchers to Co-Lead $12.9M Study (April 28, 2026)

[17] Duke Chronicle — Duke CTSI awarded $69 million NIH grant (September 2025)
